05 — Staff Training Plan

Learning objectives

After training, staff will be able to: 1. Explain why every patient is screened the same way (equity and reliability). 2. Determine whether a screen is due, using the 90-day rule. 3. Administer Q1–Q14 verbatim, with trauma-informed language and correct skip logic. 4. Score each domain correctly with the scoring key. 5. Ask about desire for help and make the right referral type. 6. Follow the safety pathway, including privacy and reporting obligations. 7. Complete the screening and referral logs accurately.

Format

Session	Length	Audience	Method
1. Why and what	30 min	All staff and volunteers	Short talk + baseline data + patient-advisor perspective
2. Hands-on administration	45 min	Anyone who screens	Demo → paired role-play with 3 scripted cases → feedback
3. Safety pathway	20 min	Screeners + clinicians	Case discussion; reporting rules from the compliance lead
4. Logs and follow-up	20 min	Screeners + referral owner	Practice filling in the logs from sample forms
Huddle refreshers	5 min	All	At the start of each clinic session during PDSA cycles

Total: about 2 hours. It can run as one block or be split across clinic days. Add it to new-staff and volunteer onboarding for sustainment.

Trauma-informed communication (key points)

- Explain why you are asking, and that the patient can skip any question.
- Use neutral, non-judgmental phrasing, for example “Where did you sleep last night?” rather than “Are you homeless?”
- Do not promise outcomes you cannot deliver. Say “I can connect you with…” instead of “They will give you…”.
- Respect a “no”. Offer the resource card and let the patient know they can ask at any visit.
- Make sure the space is private before asking safety questions.

Role-play cases

1. **Ms. A:** 54 years old, sleeping in her car, food insecure, missed two appointments for lack of a ride, and has Medicaid. → Housing routing (vehicle), food, NEMT.
2. **Mr. B:** 19 years old, staying with friends, lost his wallet with his ID and Social Security card. → Free youth credentials (< 21), SS card replacement, housing at-risk.

3. **Mx. C:** in a shelter. During the safety items, a partner is waiting nearby. → Defer the safety items, document “deferred: no privacy”, and set up a private retry.

Competency check (observed, per screener)

#	Item	Met?
1	Uses intro script	☐
2	Reads scored items verbatim	☐
3	Applies skip logic (Q2, Q6) correctly	☐
4	Ensures privacy before Q7–Q10	☐
5	Scores all domains correctly	☐
6	Asks desire-for-help for each positive domain	☐
7	Chooses appropriate resource and referral type	☐
8	Records contact method (Q14)	☐
9	Completes logs without errors	☐

Pass = 9/9 on one observed real or simulated screen. Re-observe if fewer than 9.

Training evaluation

- Pre/post knowledge quiz (8–10 items covering eligibility, scoring, and the safety pathway).
- Confidence rating (1–5) for “I feel confident screening for social needs”, before and after.
- % of screeners who pass the competency check before go-live (target 100 %).

Materials checklist

- ☐ Laminated job aid (workflow + scoring key), see 03
- ☐ Printed forms (templates/screening-form.md)
- ☐ Resource directory and patient resource cards (04)
- ☐ Interpreter-line instructions
- ☐ Knowledge quiz and competency checklist